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INDIAN INSTITUTE of MANAGEMENT AHMEDABAD IIMA/ECO0372

# Dr Lal PathLabs: Sustaining Growth in a Changing Diagnostic Landscape

## Introduction

By 2026, Dr Lal PathLabs (DLPL) had completed more than 75 years of operations and established itself as one of India's most trusted diagnostic networks, built on an aggressively executed hub-and-spoke model, a highly structured and scalable franchise ecosystem and continuous service innovation in areas such as accurate and quality reporting, extensively trained laboratory and field workforce including phlebotomists, home collection service, digital booking, rapid turnaround time and easily accessible IT-enabled laboratory reports. While its organic expansion over decades resulted in deep dominance in north, central and east India, its presence in western India was significantly strengthened through the acquisition of Suburban Diagnostics and its footprint in south India was continuously increasing.

Even as the company's financial performance remained strong, the leadership faced renewed questions about the next phase of growth. Should the DLPL continue to rely on its proven organic model of expanding from major cities into surrounding Tier-2 and Tier-3 towns through its franchise and home-collection network, or accelerate its national presence through further inorganic acquisitions in under-represented regions such as the west and south? At the same time, the diagnostic industry itself was being redefined. Alongside physician-directed testing, a new stream of self-initiated, consumer-driven demand was emerging, influenced by preventive health awareness, digital platforms and wearable technologies. For complex and chronic conditions, diagnostic journeys were no longer confined to pathology alone. They now increasingly combined blood tests with ultrasound, computed tomography (CT), magnetic resonance imaging (MRI) and other advanced screening modalities, domains where hospital-based providers held a structural advantage.

As the DLPL reflected on its position within this shifting ecosystem, a two-fold question began to surface: How could it scale geographically, serve both prescription-based and self-initiated consumers and determine whether to remain a pathology specialist or evolve into a more integrated diagnostic platform with CT and MRI for the future?

## Background and evolution

### 1949–1976: Early years

The DLPL traces its origins to 1949, when Dr Major S. K. Lal, a pathologist trained in the Indian Army Medical Corps, founded a small diagnostic facility in Delhi as the Central Clinical Laboratory and a blood bank known as the Blood Transfusion Centre. Born in Rawalpindi and graduated from King Edward Medical College in Lahore, Pakistan, Dr S. K. Lal carried

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this formative education into the laboratory's early years, guided by his experience as an Army pathologist, and his belief that diagnostic medicine must uphold accuracy above all else. In an era when pathology was largely manual and dependent on personal expertise, he built a standardised practice grounded in methodical processes, reliability and professional integrity.

Through the 1950s and 1960s, the laboratory earned clinicians' trust by maintaining meticulous operating standards and introducing new tests as technologies. With no laboratory automation in India at the time, quality depended heavily on the laboratory's internal procedures, documentation practices and the pathologist's technical competence. These decades cemented the DLPL's working style: scientific rigour, a reputation built on consistency rather than scale and a relationship-driven model where clinicians formed the core referral channel.

After Dr S. K. Lal passed away in 1977, his son Dr Arvind Lal, supported by his wife Dr Vandana Lal and his mother Mrs Vimla Lal, took over. For nearly three decades, this trio navigated the operational, emotional and strategic dimensions of building the enterprise together.

**1977–2005: Introduction of franchising, hub-and-spoke model, pioneering new artefacts and digitisation of laboratory practices**

The period following the late 1970s marked a decisive transformation in the laboratory's operating model. As demand for diagnostics increased, the company broadened its testing capabilities and upgraded its equipment, moving from earlier semi-automated processes towards modern automated systems (for example, thyroid tests in conjunction with the first automated blood chemistry autoanalyser in north India were initiated in 1981). These transitions required both capital and conviction. As Dr Arvind Lal recalled, "In 1982, I went to the Canara Bank, adjacent to our laboratory, to ask for a loan for the first time, for a princely amount of ₹3 lakh. My mother was scared, but ultimately gave in and that loan helped me take the lab operations to the next level with automation."

With growing volumes, the company began adopting modern instruments and techniques, such as vacutainers (the DLPL was the first in India to adopt them in practice), improved analysers, barcoding systems and computerised reporting long before such practices became commonplace. These improvements strengthened internal quality systems and created consistency across workflows. As test volumes expanded, it began to experiment with a structure in which samples were collected across multiple smaller touchpoints. Complex and rare tests required centralised equipment and oversight, whereas sample collection could occur closer to the patient. This backend-centralised, frontend-distributed architecture laid the foundation for the hub-and-spoke network, reducing capital expenditure duplication, ensuring consistent quality control and creating a scalable operational template.

Franchising emerged in 1982 through a practical challenge. Difficulty in securing rented premises for collection centres led to the creation of a revenue-sharing arrangement in which the landlord provided the centre for sample collection, while the laboratory provided testing and brand support. This model aligned incentives for both parties, kept expansion costs low and became the blueprint for neighbourhood-level access across India.

Digitisation followed soon after. As patient volumes increased from approximately 30 a day in 1977 to nearly 90 by the mid-1980s, the need for operational efficiency became evident. In

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1986, the DLPL developed India's first healthcare software, Laboratory Information Management System (LIMS), enabling full computerisation of laboratory workflows. Automated highlighting of abnormal values, standardised reference ranges and consistent reporting formats replaced the manually typed reports that had previously constrained scalability.

The late 1980s brought further technological firsts. In 1989, the laboratory became the first private facility in India to offer HIV/AIDS testing. The same year, it introduced the Hitachi 717 biochemistry autoanalyser, the first installation of its kind in India, capable of processing approximately 600 samples per hour. In 1994, a collaboration with the Corning Clinical Laboratories (now Quest Diagnostics) enabled Indian patients to access high-end tests performed in the United States, with billing in rupees following economic liberalisation.

In 1998, the Australian chain Gribbles Pathology entered India, prompting the DLPL to expand its network to protect its regional base. In 1999, the company adopted Ultra LIMS, connecting remote laboratories with the central facility in Delhi via the Insat-3C satellite. Soon afterwards, it installed the Hitachi 917 autoanalyser, which delivered up to 800 tests per hour. Samples from decentralised collection points were transported in cold chain maintained boxes to the central reference laboratory for complex testing, a backend-centralised, frontend-distributed logic that evolved into a mature hub-and-spoke network (see Exhibits 1.1, 1.2, 1.3), yielding cost efficiency and standardisation at scale.[1]

In 2003, the DLPL declined a proposed 50% joint venture with Quest Diagnostics when the latter sought to enter India directly. Quest subsequently established its own operations, but struggled to gain traction and later exited the market. By 2005, the DLPL had evolved from a respected single-city laboratory into a structured regional network underpinned by automation, two generations of LIMS, franchising and a mature hub-and-spoke architecture.

**2005–2015: Transition from family to professional management**

The mid-2000s represented a key inflection point as the DLPL transitioned from a family-led company to one with professional leadership and enhanced institutional governance. Like many family businesses, the DLPL's promoters found themselves at crossroads of control and scale in 2005–2006—the company's growth required a more structured approach where professional managers and outside capital would play a prominent role. The decision in favour of scale, relinquishing some family control, significantly altered the DLPL's growth trajectory.

Scale needed capital infusion. As Dr Arvind Lal observed, "I understood very early that in order to scale we needed two kinds of finance: first debt, second equity." The DLPL hired its first chief financial officer, who advised that "equity is the answer" for sustaining expansion. Private equity investments by Bengaluru-based WestBridge Capital in 2005 and TA Associates in 2010 brought greater performance discipline and governance oversight, without displacing the company's clinical ethos.

Dr Om Manchanda joined the DLPL in 2005 as chief operating officer, became the chief executive officer in 2008 and then managing director in 2020. When he joined, the DLPL's

[1] In this 2019 interview, Dr Manchanda explained that the network of collection sites and labs would yield cost efficiency and standardisation at a larger scale. See EY India. (2020, February 23). EOY India 2019 "Life Sciences & Healthcare" category winners Dr. Arvind Lal & Dr. Om Manchanda. YouTube. https://www.youtube.com/watch?v=PvG2K-7VR28, accessed April 7, 2026.

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turnover was approximately ₹43 crore and its enterprise value (pre-money) was ₹80 crore. Coming into a family-led business after spending most of his career in large, professionally managed firms, Dr Manchanda's mandate was to help redesign the company as it prepared for scale. He recalled, "[T]he first thing that I had to do was to really put a management structure in place from an owner-driven company." By 2025, the company scaled to nearly ₹2,500 crore in revenues and a market capitalisation exceeding ₹20,000 crore.

The diversification of executive ranks introduced two very different organisational cultures, deeply service-oriented medical professionals alongside commercially trained managers with significantly different compensation structures, expectations and decision-making styles. Despite these differences, the transition unfolded smoothly because the incoming professionals shared the founders' vision: they wanted to scale, sought external capital and supported the idea of private equity. Dr Arvind Lal had understood early on that the company was ready for professionals who would challenge ideas and contribute independently. As Dr Manchanda observed,

> [F]rom a startup stage to a business that is about to scale up, a lot of companies suffer from letting go and delegating things … and that really suffocates the company's ability to grow…. For scale to take root leadership had to shift its focus so that the right decision could be made.

At the DLPL, professional managers were encouraged to challenge ideas constructively.

The period from 2010 to 2015 was the DLPL's significant growth phase, with the company expanding hospital partnerships, developing backend capacity and implementing systematic operational processes, moving from Delhi to Bareilly, Moradabad and other parts of north India, steadily building strong regional dominance.

**2015: The IPO and institutionalisation phase**

The 2015 initial public offering (IPO) on the National Stock Exchange of India and Bombay Stock Exchange marked the culmination of the DLPL's decade-long institutionalisation. The Draft Red Herring Prospectus[2] described the company as one of the largest and most trusted diagnostic service providers in India with an extensive network of laboratories and patient service centres across multiple regions.[3] Between FY2016 and FY2020, the DLPL more than doubled its revenues and footprint through process standardisation, data-driven decision-making and regional diversification. Its hub-and-spoke and franchise-based model allowed operational consistency across geographies, even as the broader industry remained dominated by thousands of unorganised standalone diagnostic centres.

Dr Manchanda, emphasised bringing the laboratory closer to the patient rather than forcing the patient to come there. By end 2015, the network had expanded to over 172 clinical labs, 1,559 patient service centres (PSCs) and 4,967 pick-up points (PUPs), supported by automation and real-time tracking. This entire network provided the backbone for high-volume efficiency across Tier-1 to Tier-4 cities. The company's emphasis was on logistics efficiency, quality

[2] A Draft Red Herring Prospectus is a preliminary offer document filed with the Securities and Exchange Board of India prior to an IPO, containing mandatory disclosures on the company's business model, financials and risk factors.

[3] Dr Lal PathLabs Limited. (2015, September 12). Draft red herring prospectus.

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control and consistent turnaround–time-enabled scalability without compromising quality, something that most fragmented competitors struggled to replicate.

**2020 to present: COVID-19 and beyond**

The COVID-19 pandemic became a defining event for the entire industry, which transformed diagnostics from a background service into a frontline healthcare necessity. For many, this represented an opportunity to expand, although this was contingent on the management of existing company resources. For instance, smaller laboratories struggled to cope with rapidly rising demand and testing standards, while organised players with stronger operational depth scaled with far greater agility.

For the DLPL, the pandemic created an urgent and immediate push to expand capacity. Several state governments, including Rajasthan, Haryana and parts of Uttar Pradesh, relied on the company for high-volume reverse transcription polymerase chain reaction (RT-PCR) processing. The DLPL was able to ramp up operations within months, testing nearly 35 lakh RT-PCR samples in 2021 alone, a feat made possible by the extraordinary effort of its employees. Dr Manchanda noted that COVID-19 reframed leadership away from numbers and towards purpose, stating that every business seeks a higher purpose, making it easier to mobilise teams by reminding them that "you are serving humanity". Nearly half of the DLPL's workforce was affected by the virus at different points, yet operations continued uninterrupted, reflecting both operational resilience and internal alignment. Dr Arvind Lal observed, "In dire medical emergencies, like the current COVID-19 pandemic, the testing load goes up exponentially and reliable private diagnostic providers are needed to help the government in augmenting their testing capabilities."

In the 2021–2022 annual report, Dr Manchanda wrote, "The pandemic underscored the strength of our network and validated our digital readiness. We responded with agility, scaling molecular testing while ensuring safe access to diagnostics."[4] The DLPL managed unprecedented testing volumes while accelerating digital adoption through e-booking, online reporting and home collection, with millions of first-time users interacting directly with its services.

The acquisition of Suburban Diagnostics in 2021 introduced post-merger integration complexity alongside navigating COVID-19. Though a risky decision given the uncertainty, the move was anticipatory, expanding to western India—Suburban Diagnostics had a strong base in Mumbai—ahead of expected industry consolidation. As testing demands normalised, the DLPL continued its franchise-based model to drive organic penetration into Tier-3 and Tier-4 markets, blending organic and inorganic expansion. The 2022–2023 annual report noted that "the demand for diagnostic services is experiencing a significant surge, driven by growing awareness about health and wellness, lifestyle disorders, and an aging population."[5]

Technology modernisation became central to its growth story. The 2023–2024 annual report emphasised the company's progress along "three strategic focus areas—geographical expansion, portfolio diversification, and digital transformation".[6] Dr Arvind Lal observed that "digital transformation is no longer an aspiration but a driver of competitive advantage". The DLPL pioneered artificial intelligence- (AI) based histopathology and digital pathology for breast and prostate cancer, becoming one of India's first diagnostic networks to integrate

[4] Annual report 2021–22, DLPL.
[5] Annual report 2022–23, DLPL.
[6] Annual report 2023–24, DLPL.

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machine learning tools in image analysis, and launched Reco.ai and Chatbot 2.0 for continuous patient engagement. Its "Swasthfit" preventive health programme became a flagship offering, and the 2024–2025 annual report noted research collaborations with Indian Institute of Technology Delhi on AI-driven histopathology and computational diagnostics, marking the DLPL's entry into the precision medicine space. By FY2024–2025, the DLPL had served tens of millions of patients annually while maintaining strong margins and operational discipline.

Non-communicable diseases (NCDs) or lifestyle diseases accounted for a growing share of morbidity and mortality in India. Nearly 65% of all deaths in India are now caused due to NCDs, responsible for nearly 15,000 deaths every single day, with an annual gross domestic product loss of around 2%. Major NCDs include heart disease, diabetes, hypertension, stroke, cancers and chronic lung, liver and kidney diseases, particularly acute in rural areas where awareness, early detection and access to organised diagnostics remained limited. Recognising this gap, Dr Arvind Lal proposed GraminFit, a compact diagnostic package for early risk identification in rural populations. The DLPL partnered with the National School of Drama to conduct village-level street plays explaining lifestyle diseases and the importance of early testing, while mobile phlebotomy units were deployed at weekly *mandi*s for on-site sample collection. By 2025, the initiative was scaled through the DLPL's franchise network, reaching over 2 lakh patients.

In January 2026, the DLPL further reinforced its commitment to structured preventive healthcare with the launch of its first diagnostic-led wellness centre, Sovaaka, in Gurugram, Haryana, designed to help individuals understand their health from the inside out—before symptoms appear, before risk turns into disease. Under one roof, Sovaaka offers comprehensive whole-body diagnostics, advanced imaging with low radiation exposure, genetic insights, face-to-face doctor consultations and personalised nutrition and lifestyle guidance. Every recommendation at Sovaaka is clinically aligned, evidence-based and personalised. It includes pre- and post-doctor consultations, a smart report with clear interpretation of results and actionable guidance, empowering individuals to proactively manage their long-term health.

As the company entered its 76th year, it stood as a phygital, tech-enabled healthcare institution balancing its clinical heritage with modern consumer expectations. Its size, profitability and brand trust were unmatched, yet the next phase demanded a more deliberate balance between organic depth, selective inorganic expansion, digital innovation and strategic focus within a largely unorganised and rapidly evolving diagnostic landscape.

**Growth and organisational complexity**

Over the decade following its 2015 listing, the DLPL experienced a pattern of growth typical of a company that had successfully institutionalised its first phase of professional management. Between FY2015 and FY2025, the company's revenue expanded by approximately three times, while margins improved steadily, with operating profitability rising extensively over the same period (see Exhibit 2). Viewed over a longer arc, the trajectory was even more striking. Over the past two decades, turnover rose from ₹43 crore in 2005 to ₹660 crore in 2015 and ₹2,461 crore in 2025. By FY2025, the company reported cash and cash equivalents of over ₹1,200 crore.

Among organised diagnostic players, this scale and profitability were unusual. Most laboratories in India remained small, founder-dependent and operationally constrained, often

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unable to move beyond limited geographies or informal systems. In contrast, systematic investment in professional management, technology and operational discipline enabled the DLPL to convert clinical capability into scalable infrastructure. Reflecting on the managerial foundations of this scale, Dr Manchanda observed,

> If you plot the price trends you will actually find in the last 20 years, in real terms, prices have actually been falling very sharply. What has made that happen is that we built a scale which is beneficial to all stakeholders. Prices went down, accessibility went up, quality went up and you have a professional management team running the company.

This emphasis on scale as an organisational capability rather than a numerical outcome allowed the company to translate growth into lower unit costs, broader access and consistent quality. However, this growth was not evenly distributed geographically. A substantial share of revenue came from north and east India (see Exhibit 3), where the company had strong organic roots built over decades. Presence in west India, particularly in metropolitan markets such as Mumbai, was strengthened through the acquisition of Suburban Diagnostics, while the footprint in south India remained comparatively less.

From the early years post listing, the company consciously shifted towards consolidating backend operations while expanding frontend access points. Establishing full-scale laboratories was capital-intensive, both in terms of real estate and high-value equipment. Instead, the DLPL aggressively executed a hub-and-spoke model, centralising advanced testing capabilities while scaling through a dense franchise-based network of patient service centres and home collection units.

One advantage of this approach was that rare and specialised tests could be processed at central laboratories while keeping customer access highly distributed. It also expanded the test menu without proportionately increasing laboratory footprint (see Exhibit 3), a structural advantage that many competitors were unable to replicate (see Exhibit 4). Expansion was further reinforced by service innovation. While pathology remained the company's core capability from the beginning, it was the professional management that reoriented operations around the customer. A key antecedent of the DLPL's competitive advantages was their early transformation from a pure family business to a more consolidated professional corporation. As mentioned earlier, many laboratories often hesitate to bring in professional managers and external executives as, in the trade-off between control and expansion, they prioritise control. Additionally, their predominant doctor/health care specialist culture sustains or improves their technical quality, but hinders improvement in management and operations. Technological capability improvements taper off as most of the digital functions of the laboratory are outsourced to vendors and remain stagnant after the initial installation. For the DLPL, however, digital appointment booking, doorstep sample collection, rapid reporting and transparent communication transformed diagnostics into a consumer-facing experience. Home collection, in particular, drove volumes and brand visibility, building trust and convenience simultaneously.

As scale increased, so did ambition. Once national leadership was established in the north and east, attention turned to regions where the company had limited natural presence. Organic entry into west and south India required time and the slow building of referral relationships, physician trust and operational density. Inorganic routes provided faster market access, but came with integration risks in systems, people and regional culture. Prominent executives echoed both organic and inorganic expansion as viable routes. For instance, Dr Manchanda

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said in a 2024 interview, "We will go for organic expansion in north and eastern parts of the country, where we are strong. In the western and southern markets, where we are relatively weak, we will go for both organic and inorganic growth."[7] A similar sentiment on inorganic expansion was echoed by Shankha Banerjee,[8] mentioning the possibility of mergers and acquisitions in the southern part of the country. Having conducted both types of expansion, senior leadership repeatedly observed that organic growth preserved cultural alignment, while acquisitions accelerated presence, but required disciplined post-integration management to realise value.

Beyond systems and culture, differences in quality infrastructure across regions also shaped expansion complexity. While the DLPL's core laboratories operated under National Accreditation Board for Testing and Calibration Laboratories (NABL) accreditation, the broader diagnostic landscape in many Tier-2 and Tier-3 cities was more fragmented, with several standalone laboratories functioning without formal accreditation. When the DLPL builds its own new facilities, it sets quality standards from the ground up, ensuring NABL compliance as part of the setup. The more nuanced dilemma arose with franchise partnerships and acquisitions, both of which involved engaging with existing third-party operators or facilities that may not meet the DLPL's quality benchmarks. In such cases, the company would need to determine whether to restrict partnerships and acquisitions to already-accredited facilities or invest in upgrading processes, documentation, equipment calibration and staff training to align them with the company's vision. In practice, extending the DLPL brand over substandard facilities carried direct reputational risk, making compliance support a cost the company typically had to absorb or direct as a condition of onboarding. In a market where physician trust and scientific positioning were central to competitive advantage, decisions about how far to extend the DLPL brand without compromising quality became strategically consequential.

Organisational complexity also increased. The company evolved into a multilayered operation spanning hundreds of laboratories, thousands of patient service centres and a geographically dispersed workforce. While the integration of Suburban Diagnostics introduced variation in systems and work culture, leadership continuity remained stable. The challenge was not turnover, but harmonisation. Standardisation, once the backbone of efficiency, had to be balanced with regional sensitivity, as patient behaviour, physician engagement and service expectations varied across geographies.

Certain limitations remained evident. Advanced imaging, including CT, MRI, ultrasound and specialised radiology, involved larger capital commitments, specialised staffing, constant technological upgrades and limited feasibility as preventive tests. Unlike pathology, imaging results were difficult for consumers to interpret without physician input. This made integration into a consumer-led model structurally challenging. While providers that had originally grown from imaging and radiology were steadily extending their services into pathology, the reverse was slightly complex. At the same time, genomic testing was also being discussed within the company and among physicians.

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[7] Moneycontrol News. (2024, May 21). Firms need private equity money to grow, but can't expect overnight success: Dr Lal PathLabs' Arvind Lal. *Moneycontrol*. https://www.moneycontrol.com/news/business/companies/firms-need-private-equity-money-to-grow-but-cant-expect-overnight-success-dr-lal-pathlabs-arvind-lal-12728181.html, accessed April 10, 2026.

[8] Pilla, V. (2024, October 1). Dr Lal PathLabs sets sights on big buys to expand in south. *The Economic Times*. https://economictimes.indiatimes.com/industry/healthcare/biotech/healthcare/dr-lal-pathlabs-sets-sights-on-big-buys-to-expand-in-south/articleshow/113827444.cms, accessed April 10, 2026.

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Investor expectations evolved accordingly. While earlier phases of growth were driven by strong performance in established geographies, recent investor discussions increasingly emphasised the need for calibrated expansion into underpenetrated regions, along with deeper test specialisation and sustained operational efficiency. The focus shifted from rapid geographic replication to a more measured, risk-aware approach to long-term scalability.

Each pathway implied a different organisational configuration and capital commitment. The central challenge, therefore, was no longer simply about expansion, but about coherence. It was a question of how far and in what form the company could stretch without compromising the service quality, discipline and trust that had enabled its success.

**Evolving consumer expectations in diagnostics**

The Indian diagnostic market was undergoing a profound transformation that reshaped how individuals engaged with healthcare. For decades, testing had largely been a doctor-directed process, where patients followed prescriptions and laboratories functioned primarily as institutional service providers. After 2020, however, a new layer was added to this structure. Rising awareness of preventive health, increasing lifestyle-related NCDs and the spread of affordable smartphones created a growing segment of individuals who initiated testing on their own, even in the absence of immediate medical consultation, reflecting a broader shift towards consumer-driven and preventive healthcare engagement.[9,10] Rather than replacing physician-directed testing, this self-initiated behaviour coexisted, gradually forming a parallel stream of demand.

The pandemic accelerated this shift. Millions of patients experienced diagnostics directly through home collection, app-based booking and online result delivery. A previously institutional interaction became part of a broader consumer experience, influenced by convenience, speed and price transparency. The market saw an influx of digital-first health platforms such as Tata 1mg, Healthians, PharmEasy, Netmeds, Apollo 24x7, Practo and Amazon Diagnostics, offering bundled tests, app-based booking and recurring health packages. These platforms were built around mobile-first interfaces, algorithmic recommendations and seamless user experience rather than physical collection networks or traditional prescription systems. In many cases, individuals who monitored their health through wearables and basic home-testing devices such as blood glucose monitors, digital blood pressure machines and smartwatches chose to visit diagnostic centres periodically, such as once every six months or a year. If abnormalities were detected, they later approached physicians with their prior reports, who in turn directed them towards appropriate paths, be it treatment or further tests.

For the DLPL, this evolving behaviour represented both an opportunity and a structural challenge. The brand stood for reliability and clinical precision, positioning it well to attract consumers seeking trust in an increasingly crowded and promotional marketplace. At the same time, its systems and processes had been historically oriented towards a physician-driven model, where institutional relationships and professional referrals played a central

[9] Grand View Research. (n.d.). Home care testing market (2025–2030). https://www.grandviewresearch.com/industry-analysis/home-care-testing-market-report, accessed April 10, 2026.

[10] Market Growth Reports. (2025, December 10). At-home health testing platform market size, share, growth, and industry analysis, by type (general health, men's health, women's health, infectious disease detection), by application (consumer-grade diagnostics, clinical grade diagnosis), regional insights and forecast to 2035. https://www.marketgrowthreports.com/market-reports/at-home-health-testing-platform-market-101646, accessed April 10, 2026.

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role. The new self-initiated consumer was often younger,[11] more digitally fluent and motivated by prevention, wellness and personal data. In contrast, the traditional, prescription-driven customer base tended to be older, diagnosis-focused and more dependent on clinical guidance. As Dr Manchanda put it, "At the consumer level the behaviour is changing. Earlier, it used to be more prescription-driven testing. Now it's more health checkups, screening tests which are not really prescription-driven." Meeting these divergent expectations required not only technological investment, but also a more nuanced understanding of how different customer segments now interacted with diagnostics at different points in their health journey.

The company's growth in preventive and wellness packages illustrated this shift in demand. Programmes such as Swasthfit reflected a market in which individuals were increasingly purchasing diagnostic services as part of ongoing lifestyle and risk management rather than only in response to illness. Reports suggest that preventive healthcare in India has grown steadily in recent years, driven by rising awareness, increasing expenditure in health and wellness and a broader shift towards early detection and health tracking (see Exhibit 5). At the same time, pricing strategies and promotional campaigns by digital aggregators began to influence customer loyalty. Repeat engagement now depended not just on accuracy, but also on experience, accessibility and perceived value over time.

A further complexity lay in the expanding definition of diagnostics itself. As healthcare became more integrated, the boundary between pathology, radiology and advanced screening began to blur. For many conditions, blood tests alone were no longer sufficient. Diagnostic pathways increasingly required a combination of pathology, radiology and imaging modalities such as CT, MRI and ultrasound, visible in areas such as oncology, where histopathology was interpreted alongside radiological scans, and in maternal care, where ultrasound formed the backbone of routine pregnancy monitoring. Hospitals were able to offer this entire continuum under one roof, supported by in-house specialists and capital-intensive equipment.

Expanding into imaging and advanced diagnostics, however, presented significant challenges. The financial investment required for machines, skilled radiologists, maintenance and infrastructure was substantially higher than in pathology. At the same time, radiology accounted for a substantial share of the overall diagnostic market, reflecting its scale and relevance within the industry (see Exhibit 6). In radiology, equipment lifecycle is closely linked to technological relevance. Many professional standards indicate that at least 60% of installed imaging equipment is typically expected to be less than five years old[12] in order to be considered aligned with current technological benchmarks. In effect this was a capital-intensive venture. Remaining outside this integrated pathway risked ceding control of the patient journey to competitors who could capture the entire diagnostic bundle, not just an individual test.

Adapting to this environment required the company to view its future not as a single business model, but as a portfolio of diagnostic solutions serving different entry points to healthcare. One segment remained firmly rooted in physician prescriptions and clinical pathways. Another emerged from self-initiated, technology-enabled and prevention-focused consumers. Balancing these segments, while keeping pace with rapid technological change, required

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[11] Cecconi, C., Adams, R., Cardone, A., Declaye, J., Silva, M., Vanlerberghe, T., Guldemond, N., Devisch, I., & van Vugt, J. (2025). Generational differences in healthcare: The role of technology in the path forward. *Frontiers in Public Health*, *13*, 1546317.

[12] European Society of Radiology. (2014). Renewal of radiological equipment, *Insights into Imaging*, *5*(5), 543–546. DOI: 10.1007/s13244-014-0345-1.

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careful investment choices, new partnerships and evolving organisational capabilities. Simply responding to shifting preferences was insufficient. The strategic task lay in designing a coherent ecosystem that connected self-awareness, professional care and advanced medical precision into an integrated, trusted pathway.

**Competition**

By the mid-2020s, the diagnostic industry had become far more complicated than the one DLPL first entered. The company was competing not only with other laboratories, but also hospitals, e-pharmacies, start-ups and even technologies that were quietly reshaping what it meant to stay healthy. Simultaneously, the industry remained highly fragmented and largely unorganised, with a significant share of testing still conducted by standalone and informal laboratories.

Direct competitors such as Thyrocare, Metropolis Healthcare, Suraksha Diagnostics and Vijaya Diagnostics had built distinctive identities and followed different strategic trajectories. Thyrocare focused on a highly automated, asset-light, low-cost model that emphasised scale and national reach through standardised test menus. Metropolis Healthcare built its image around clinical sophistication and specialised testing, positioning itself as a premium diagnostic provider in markets in western India. Over time, the DLPL was able to outpace Metropolis Healthcare in scale and network depth, with its market capitalisation growing to more than two and a half times that of the latter. Suraksha Diagnostics, with its strong base in eastern India, expanded aggressively in urban and semi-urban markets through doctor referrals and partnerships with hospitals. Vijaya Diagnostics, which began primarily as an imaging-focused company, built a strong regional presence in southern India, with nearly 80% of its revenues concentrated in Hyderabad before gradually expanding into both imaging and pathology. These organised networks competed for physician referrals, corporate contracts and the trust of middle-class families who valued accuracy and brand reputation.

In smaller towns, however, thousands of standalone laboratories continued to dominate through familiarity, convenience and incentive-based doctor referrals, reflecting the thoroughly unorganised nature of much of the sector. Many of them were family-run, with low prices and personal patient relationships. For the DLPL, they represented a different kind of competition. Competing with them was not simply a question of price, but of visibility, accessibility and the emotional comfort of being the “local lab”.

Hospitals created another layer of competition. Large hospital chains such as Apollo, Fortis and Max had expanded their in-house diagnostic divisions, treating them as both a source of revenue and a means to control patient data. These hospital laboratories often provided faster turnaround times for admitted patients and offered package deals for walk-ins. While the DLPL processed many specialised tests for hospital clients, it also faced the risk of losing routine business as hospitals invested in their own testing capabilities.

Then came the digital disruptors. E-pharmacy and telehealth platforms such as Tata 1mg, PharmEasy, Netmeds and Apollo 24x7 began by tying up with local laboratories and doctors. These firms leveraged their digital capabilities to build an all-encompassing interface which offered medical services like diagnostics with appropriate medicine delivery under one roof. They utilised algorithmic prompts to entice their pre-existing customer base who used them for purchasing medicines. In doing so, they aimed to be part of a customer’s journey from diagnostics to medicine delivery.

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At the same time, the DLPL's leadership noted that competition was also evolving as the market was getting organised. As Dr Manchanda observed,

> Today I find there are lots of organised players … because of funding coming from private equity and also players from hospital side coming in, even pharma players have entered this space…. The upside is that industry is getting organised, quality is moving up, service levels are going up … it's a level playing field…. If we run our company efficiently and the brand is trusted, we stand to gain out of that.

At the same time, a new category of competition was quietly emerging in the form of portable, easy-to-use home-testing devices. Tools such as blood sugar monitors, digital blood pressure kits, pulse oximeters and other rapid testing strips were becoming commonplace in households. For customers who only needed a one-off diagnosis or routine monitoring, these devices offered a simple, inexpensive alternative to visiting a laboratory.

Fitness and wellness apps capitalised on this shift by making health tracking a daily routine. Devices like Fitbit, Apple Watch and other wearables captured basic physiological signals such as heart rate, oxygen saturation, sleep patterns and activity levels. Over time, these tools shaped a more continuous, self-monitoring mindset, with reports suggesting that users rely on such devices to guide routine health decisions and, in certain cases, reduce discretionary physician visits.[13,14] Whether these platforms ultimately complement or compete with formal diagnostics remains uncertain. On the one hand, they could reinforce demand by prompting users to seek laboratory confirmation when their devices showed anomalies, or by nudging them towards periodic, preventive check-ups. Conversely, they carried the potential to deflect demand, particularly for routine or low-severity monitoring, by giving consumers a sense of reassurance that reduced the perceived need for frequent laboratory tests, a dynamic increasingly discussed in studies on self-monitoring and healthcare utilisation patterns.[15,16,17] The net effect depended on how deeply these apps embedded themselves into everyday health decisions and how effectively diagnostic companies integrated themselves into that digital ecosystem.

AI was the newest entrant in this space. Platforms such as ChatGPT, Google Gemini and various symptom checkers could now interpret basic medical information, helping users decide whether they should see a doctor or order a test. This early digital triage subtly influenced behaviour. If an AI model suggested that a symptom was minor, the consumer might postpone or skip testing altogether. Conversely, hints of potential risk could trigger an immediate decision to get tested. In either case, these tools shaped not only medical choices, but also the very mindset of health management. Diagnostics was becoming part of a broader digital ecosystem that encouraged users to monitor, question and act on their health in real

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[13] Landi, H. (2019, June 25). Patients will use health wearables to reduce trips to doctor: Survey. Fierce Healthcare. https://www.fiercehealthcare.com/tech/patients-will-use-health-wearables-to-reduce-trips-to-doctor-survey, accessed April 10, 2026.

[14] MD Connect Inc. (n.d.). Will patient wearables lead to fewer trips to the doctor? https://mdconnectinc.com/medical-marketing-insights/will-patient-wearables-lead-to-fewer-trips-to-the-doctor/, accessed April 10, 2026.

[15] Ickenroth, M. H. P., Ronda, G., Grispen, J. E. J., Dinant, G.-J., de Vries, N. K., & van der Weijden, T. (2010, October 13). How do people respond to self-test results? A cross-sectional survey. *BMC Family Practice*, *11*(77). DOI: 10.1186/1471-2296-11-77.

[16] Trinh, S., Skoll, D., & Saxon, L. A. (2025, April 23). Health care 2025: How consumer-facing devices change health management and delivery. *Journal of Medical Internet Research*, *27*, e60766. DOI: 10.2196/60766.

[17] Imaware. (2024, September 9). Long-term trends in at-home diagnostic testing post-pandemic. https://www.imaware.health/blog/diagnostic-testing-trends-post-pandemic, accessed April 10, 2026.

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time. For diagnostic companies, competition was therefore no longer only about precision and quality. It was also about presence in the consumer's health journey. Staying relevant meant being part of the conversation before a person even decided to get tested, in the moment when a device vibrated, an app sent a nudge or an AI system offered reassurance or concern.

As the company scaled up, the hardest part of competition was no longer identifying who it was competing against, but more about the difficulty in reaching new patients. As in the words of Dr Arvind Lal,

> We are something like an ice-breaker. Beyond the ship there is only ice, and a normal ship cannot break through. The ice-breaker has to go with all the power it has got, because our patients are on the other side. New patient acquisition is not going to be easy. But nothing has been easy in this journey.

**Regulatory environment**

While the competitive field kept expanding, the regulatory environment remained uneven. India's diagnostic industry had grown rapidly in a space that lacked strong, sector-specific oversight and continued to remain largely unorganised. Accreditation systems such as NABL and National Accreditation Board for Hospitals and Healthcare Providers (NABH) existed, but remained voluntary, thus affecting the overall quality of the diagnostic industry. Out of the nearly three lakh laboratories operating in India, less than 1% was accredited by the NABL. Patient awareness about accreditation was also low, reducing compliance pressure from customers on diagnostic providers. Enforcement was patchy and uneven, particularly outside Tier-1 cities, allowing thousands of small, unorganised players to continue operating without standardised quality audits. Several state governments, including Delhi and Maharashtra,[18,19] occasionally announced crackdowns on unregistered laboratories, but compliance remained inconsistent across regions. On standardisation of quality, Dr Arvind Lal shared,

> The Government of India enacted the Clinical Establishments Act [CEA] in 2010, but only a few states adopted it. Implementation of the CEA in all states, with mandatory registration and regulation of all healthcare facilities [hospitals, clinics, laboratories] and minimum standards for services, would filter out more than half of the three lakh laboratories as non-compliant laboratories. Then we must focus on accrediting the remaining 1.5 lakh credible laboratories. The NABL alone cannot meet this demand, so we will need more accreditation bodies and expanded assessor capacity. For COVID-19 testing, the NABL accredited a large number of laboratories in a record time, proving that scaling-up is possible. Therefore, it is time to act now to make 100% laboratory accreditation not just a goal, but a national commitment.

Other parts of the healthcare sector, however, were experiencing deeper and more visible regulation. Pharmaceutical and device pricing came under increasing scrutiny, with regulators intervening to cap prices on critical items such as coronary stents in 2017 and knee

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[18] *The Times of India*. (2024, April 9). HC notice to centre, Delhi govt over "illegal" sample collection by path labs. https://timesofindia.indiatimes.com/city/delhi/hc-notice-to-centre-delhi-govt-over-illegal-sample-collection-by-path-labs/articleshow/109145467.cms, accessed April 10, 2026.

[19] *NDTV*. (2024, July 9). Maharashtra government to bring new law to crack down on fake pathology labs. https://www.ndtv.com/india-news/maharashtra-government-to-bring-new-law-to-crack-down-on-fake-pathology-labs-6066116, accessed April 10, 2026.

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implants in 2018.[20,21] More recently, price monitoring under the Drug (Price Control) Order, 2013, tightened margins for both hospitals and drug manufacturers.[22] These interventions significantly reduced a key revenue source for hospitals that had previously depended on high-margin devices and branded medicines. At the same time, greater scrutiny on pricing and operating practices increased transparency requirements across hospital operations.

Facing compressed margins and reduced pricing flexibility in their core treatment services, many hospital chains began searching for alternative revenue streams that sat outside direct price regulation. Diagnostics became one of the most attractive areas for expansion.[23,24] By integrating in-house pathology and imaging units and offering bundled health packages, large hospital groups were able to cross-subsidise other regulated services while retaining greater control over patient data and service flows. As a result, diagnostic services increasingly shifted from a support function to a strategic profit centre within hospital business models.

The pharmaceutical industry's tightening regulations on advertising, clinical trials and ethical marketing created indirect consequences for related sectors. With drug companies operating under stricter promotional rules, consumer-facing diagnostics began to fill some of the communication space that pharmaceuticals once occupied. Patients who were increasingly aware of preventive health and wellness were now being targeted through diagnostic campaigns instead of branded drugs.[25,26,27,28] This evolving pattern brought diagnostics into the public spotlight, raising questions about pricing, accuracy and ethical marketing.

Hospital accreditation norms were also becoming more rigorous, demanding greater consistency in clinical processes, documentation and data traceability. Since diagnostics

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[20] Press Information Bureau. (2017, February 14). Mission to provide affordable, quality healthcare for all: Coronary stent prices capped, to bring down cost by about 380%. Ministry of Chemicals and Fertilizers, Government of India. https://www.pib.gov.in/PressReleasePage.aspx?PRID=1482693, accessed April 10, 2026.

[21] Press Information Bureau. (2017, August 16). Government fixes ceiling prices of knee implants; people of India to save ₹1,500 crore per annum. Ministry of Chemicals and Fertilizers, Government of India. https://www.pib.gov.in/newsite/PrintRelease.aspx?relid=170040, accessed April 10, 2026.

[22] *The Economic Times*. (2025, July 15). Govt revises prices of 71 medicines, including those for diabetes and infections: Check new rates. https://economictimes.indiatimes.com/industry/healthcare/biotech/pharmaceuticals/govt-revises-prices-of-71-drugs-including-those-for-diabetes-and-infections-check-new-rates/articleshow/122496225.cms, accessed April 10, 2026.

[23] *The Hindu*. (2015, February 3). Apollo gears up for massive expansion. https://www.thehindu.com/news/cities/Hyderabad/apollo-gears-up-for-massive-expansion/article8213143.ece, accessed April 10, 2026.

[24] Mergu, S. (2025, July 2). Hospital business, improving diagnostics segment to drive growth for Fortis Healthcare. *The Economic Times*. https://economictimes.indiatimes.com/markets/stocks/news/hospital-business-improving-diagnostics-segment-to-drive-growth-for-fortis-healthcare/articleshow/122163511.cms?from=mdr, accessed April 10, 2026.

[25] *The Times of India*. (2025, September 17). "Swasth Nari, Sashakt Parivar Abhiyaan" to promote nutrition, preventive healthcare. https://timesofindia.indiatimes.com/city/varanasi/swasth-nari-sashakt-parivar-abhiyaan-to-promote-nutrition-preventive-healthcare/articleshow/123929558.cms, accessed April 10, 2026.

[26] *ET BrandEquity.com*. (2018, July 26). Find out how you can #FightTheFever this monsoon in Metropolis' new ad campaign. https://brandequity.economictimes.indiatimes.com/news/advertising/find-out-how-you-can-fightthefever-this-monsoon-in-metropolis-new-ad-campaign/65143313, accessed April 10, 2026.

[27] *BrandWagon*. (2024, December 12). MediBuddy launches #WinOverWinter campaign to address winter health challenges. *The Financial Express*. https://www.financialexpress.com/business/brandwagon-medibuddy-launches-winoverwinter-campaign-to-address-winter-health-challenges-3690169/, accessed April 10, 2026.

[28] *Business Standard*. (2024, October 21). Star Health Insurance launches nationwide campaign on World Osteoporosis Day. https://www.business-standard.com/content/press-releases-ani/star-health-insurance-launches-nationwide-campaign-on-world-osteoporosis-day-124102100281_1.html, accessed April 10, 2026.

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formed the backbone of more than 70% of all clinical decisions, these standards indirectly pushed laboratories to upgrade equipment, automate reporting systems and adopt stricter internal quality protocols. For organised players, these developments reinforced higher levels of uniformity and trust. However, they also raised operational costs and increased compliance burdens. Smaller or unorganised operators found it increasingly difficult to meet these expectations, slowly widening the gap between formal networks and standalone laboratories.

As with other industries, digitisation of records added another set of regulatory boundaries. While laws on data security and usage were dormant, regulations were anticipated to become more rigid with rapid digitisation across the country. Due to its sensitive and vital nature, healthcare data would probably face a higher level of scrutiny and enforcement of data protection and cybersecurity regulations. The complexity of managing all the laws would only be sufficed by strong cybersecurity departments, data governance programmes and training for the same. This could add to the cost of operating in the diagnostic industry.

Together, these trends created a new kind of uncertainty for the diagnostic sector. While direct regulation of diagnostic pricing had so far remained limited, the precedent set in medical devices and pharmaceuticals suggested that price intervention was no longer implausible. There are increasing discussions on whether routine, high-volume pathology tests could eventually be subjected to ceiling prices.[29, 30] For the DLPL, such a scenario raised the possibility that a mandated price reduction on routine tests could compress margins across its largest revenue segments, lowering operating profit and earnings before interest, taxes, depreciation and amortisation (EBITDA). Since routine pathology forms a significant share of total volumes, even a modest reduction in average test realisation could disproportionately affect overall profitability, particularly in the presence of high fixed costs. At the same time, the impact would not be uniform across the industry. Organised players such as the DLPL operated at scale, and regulation might disproportionately disadvantage smaller, unorganised players. The leadership therefore faced a regulatory paradox: Would stricter pricing constrain growth and profitability, or could it ultimately strengthen organised players such as the DLPL?

**Strategic dilemma**

By 2025, the DLPL faced intertwined challenges that brought together internal strategic choices with shifting market expectations. The company's central dilemma was no longer about three parallel growth priorities, but about the mode and direction of its expansion in an increasingly uneven and segmented healthcare landscape.

One critical question was how to pursue geographic expansion. The DLPL had traditionally grown organically by extending its hub-and-spoke model outward from Delhi into smaller cities and towns across north and east India, gradually building dense networks in cities such as Bareilly, Faridabad, Moradabad and other Tier-2 and Tier-3 cities. Yet its presence in west and south India remained comparatively limited, and much of its foothold there came only through the Suburban Diagnostics acquisition. Purely organic expansion in these markets would require time, sustained investment and the gradual building of relationships from the

[29] Paul, D. (2026, February 14). States responsible for diagnostic centre pricing, referral practices: MoS Health tells Rajya Sabha. *Medical Dialogues*. https://medicaldialogues.in/news/health/states-responsible-for-diagnostic-centre-pricing-referral-practices-mos-health-tells-rajya-sabha-164523, accessed April 9, 2026.

[30] Shivadekar, S. (2025, March 19). Maharashtra government to regulate prices of diagnostic tests. *Mid-Day*. https://www.mid-day.com/mumbai/mumbai-news/article/maharashtra-government-to-regulate-prices-of-diagnostic-tests-23502338, accessed April 9, 2026.

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ground up. In contrast, inorganic expansion through further acquisitions or partnerships could provide faster access to infrastructure, volumes and brand visibility, but at the cost of hyped valuations, inter-laboratory integration complexity and potential cultural misalignment. A hybrid approach, combining organic penetration in adjacent towns and cities with targeted inorganic entry in major urban centres, appeared attractive, yet demanded careful coordination and long-term resource commitment.

Running parallel to this geographic question was a change in customer mental disposition. Diagnostic demand was no longer solely defined by physician prescriptions. A growing segment of consumers was now initiating testing independently, influenced by rising health awareness, lifestyle monitoring, wearable devices and periodic self-checks conducted every few months. Importantly, this shift was additive rather than substitutive. Physician-directed testing continued to remain essential, especially when illness was detected or treatment was required. In many cases, individuals first used self-initiated tests and preventive screenings, and later approached doctors with these results, who in turn prescribed further, more specific investigations. This created a dual stream of demand consisting of both prescription-based and self-initiated segments.

The strategic dilemma, therefore, was not whether to serve one or the other, but how to design a coherent portfolio that addressed both, without fragmenting focus or diluting quality. Serving the prescription segment required continued excellence in quality, clinical relevance and professional trust. Catering to the self-initiated segment demanded investment in digital interfaces, quick turnarounds, personalised communication and consumer-friendly packaging of services. Balancing these two streams required careful architectural and organisational choices, including decisions on branding, pricing, technology platforms and channel management. Further complexity arose from the changing scope of diagnostics itself. In many clinical contexts, pathology on its own was no longer sufficient. Conditions such as cancer, cardiac diseases, maternal care and musculoskeletal injuries increasingly required a combination of blood tests, ultrasound, CT and MRI to present a complete clinical picture. Hospitals were able to offer this integrated chain under one roof, giving them an inherent advantage in providing end-to-end diagnostic pathways.

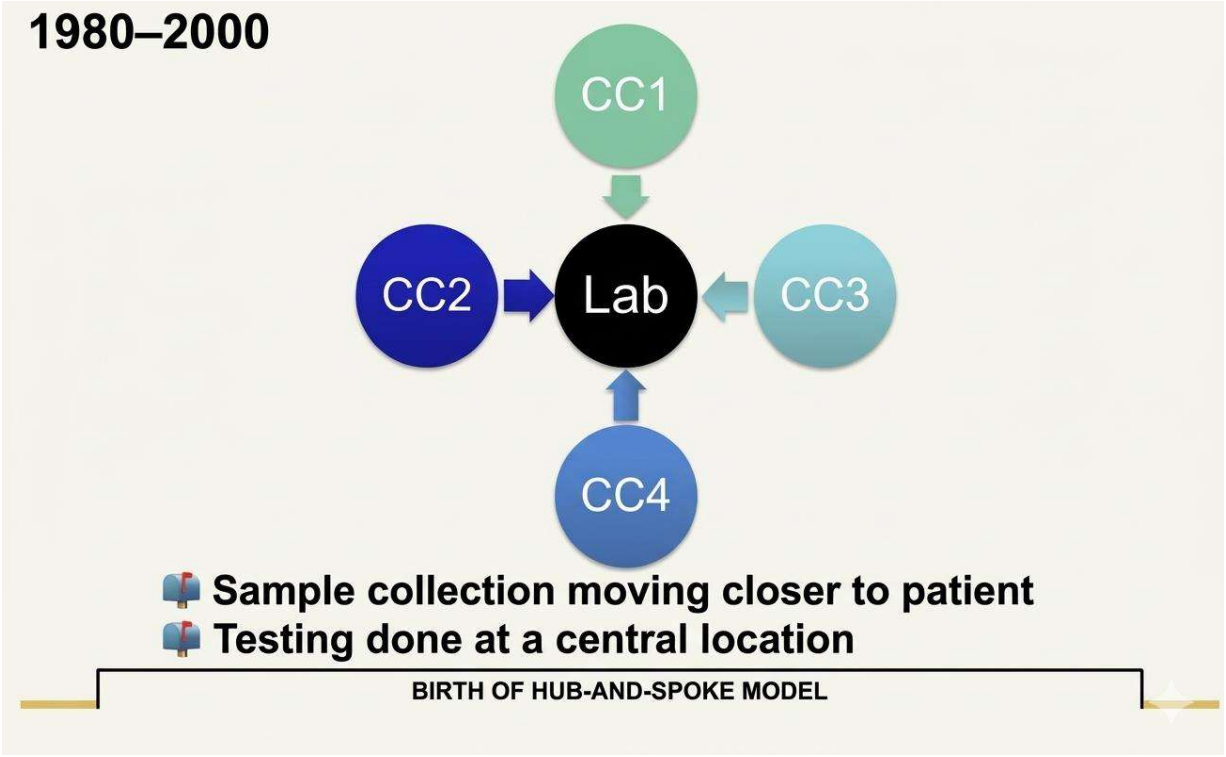
In effect, the company's strategic challenge could be framed along two intersecting dimensions. The first concerned the structure of geographic growth, that is, organic and inorganic, or a carefully sequenced combination of both. The second concerned the architecture of its diagnostic portfolio, that is, how to expand beyond traditional pathology to serve both prescription-driven and self-initiated consumers while deciding whether and how to integrate imaging and advanced diagnostics. These paths were not without compromise. Ultimately, the decisions it made would determine not only the next phase of growth, but also whether the DLPL would remain a pathology-focused leader, evolve into an integrated diagnostic platform or become a hybrid institution balancing multiple identities within India's rapidly transforming healthcare ecosystem.

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## Exhibits

### Exhibit 1.1: Pathology Laboratory Network




**Notes:** CC1 / CC2 / CC3 / CC4 = collection centres 1, 2, 3 and 4: multiple sample collection points spread across different locations.

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**Exhibit 1.2: National Pathology Grid Post 2000**

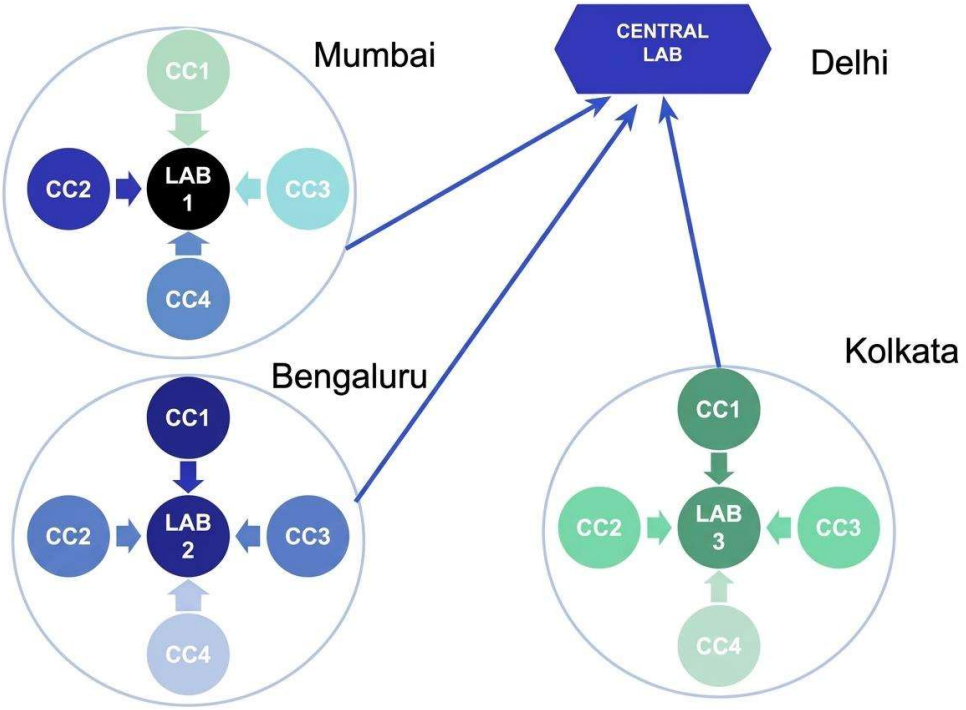
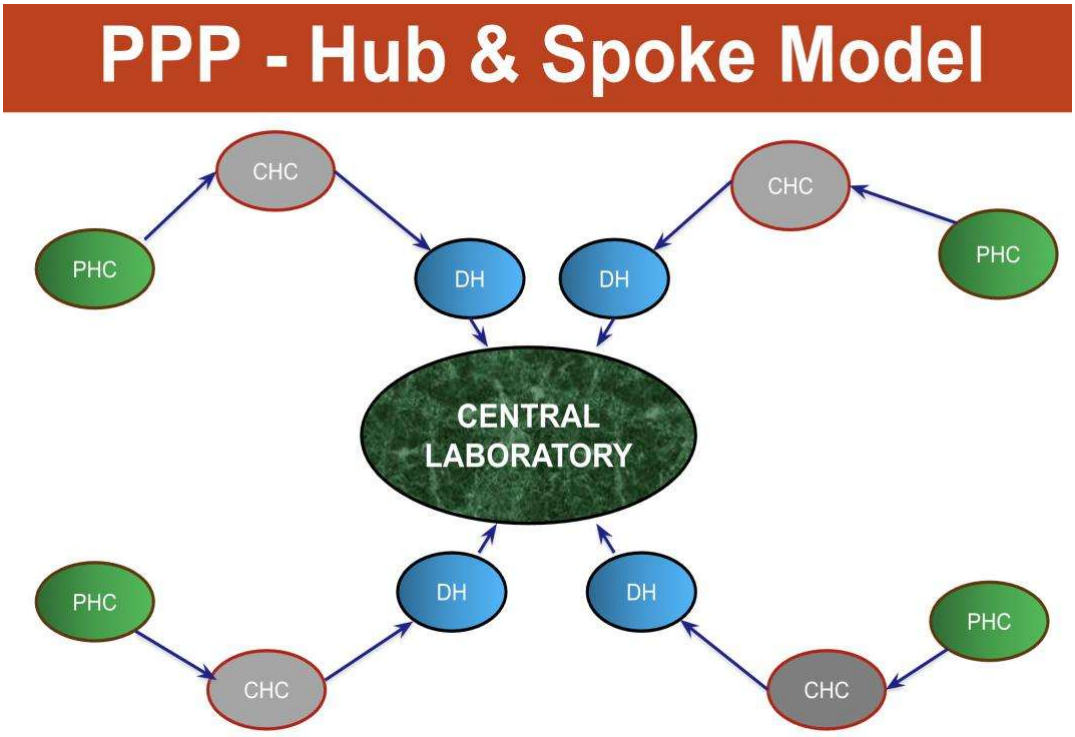



**Exhibit 1.3: PPP—Hub-and-spoke Model**



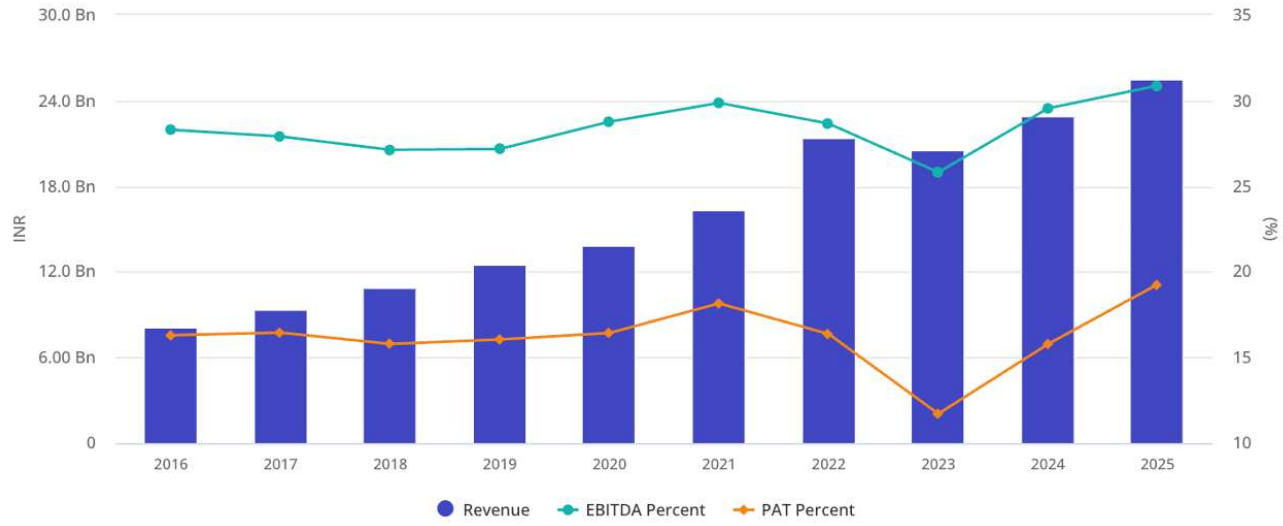

**Source:** Company presentation by Dr Arvind Lal, Dr Lal PathLabs.

**Notes:** PHC = primary health centres, CHC = community health centres, DH = district hospitals.

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**Exhibit 2: Income Statement of the DLPL**



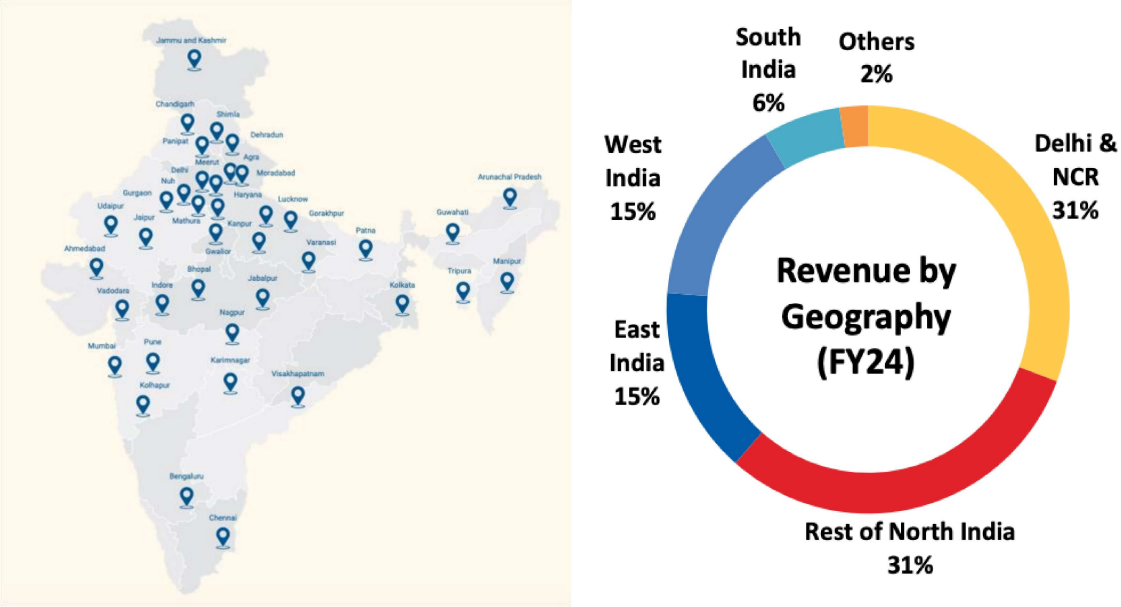

**Source:** Privatecircle.

**Note:** PAT = profit after tax.

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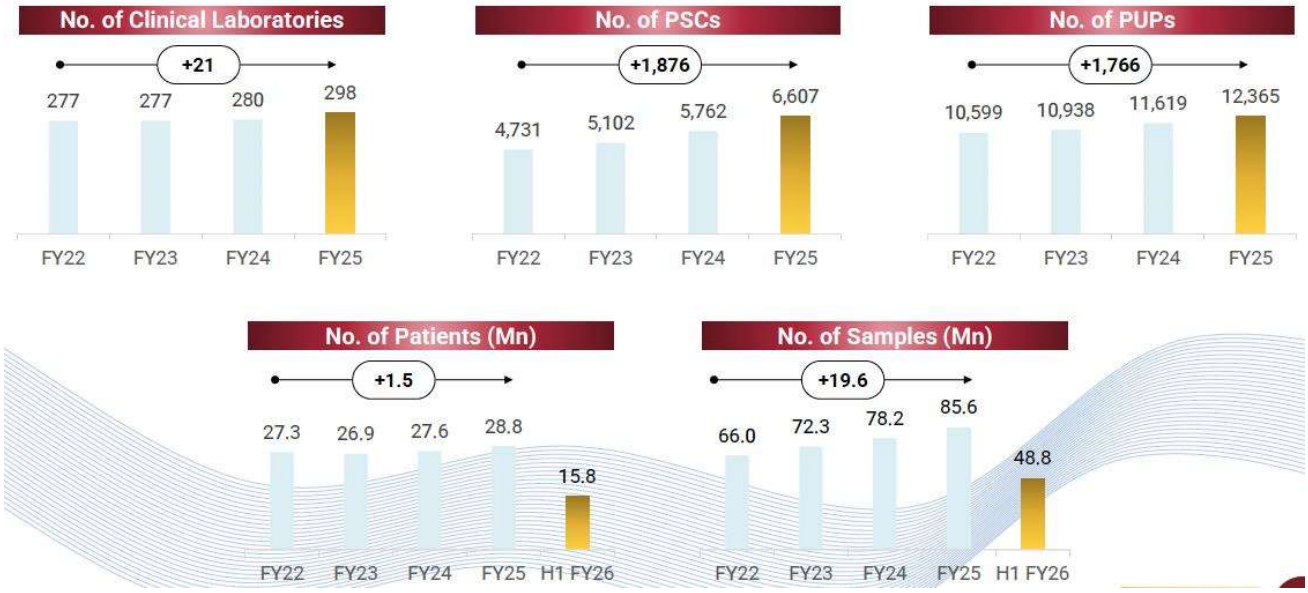


**Exhibit 3: Footprint and Revenue by Geography**




**Source:** Annual report 2024–2025, DLPL.

**Exhibit 4: Operating Highlights**

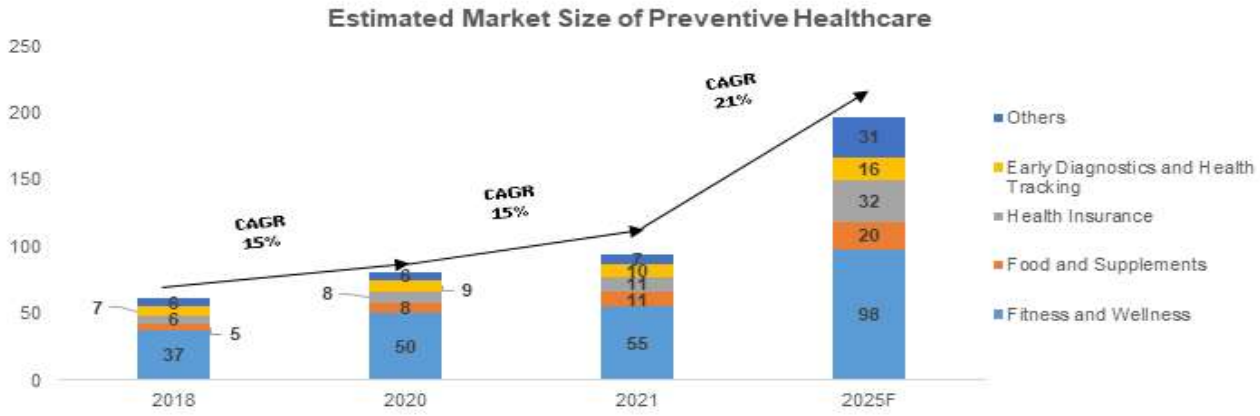



**Source:** Investor presentation, Q2 FY2026, DLPL.

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**Exhibit 5: Estimated Market Size of Preventative Healthcare**



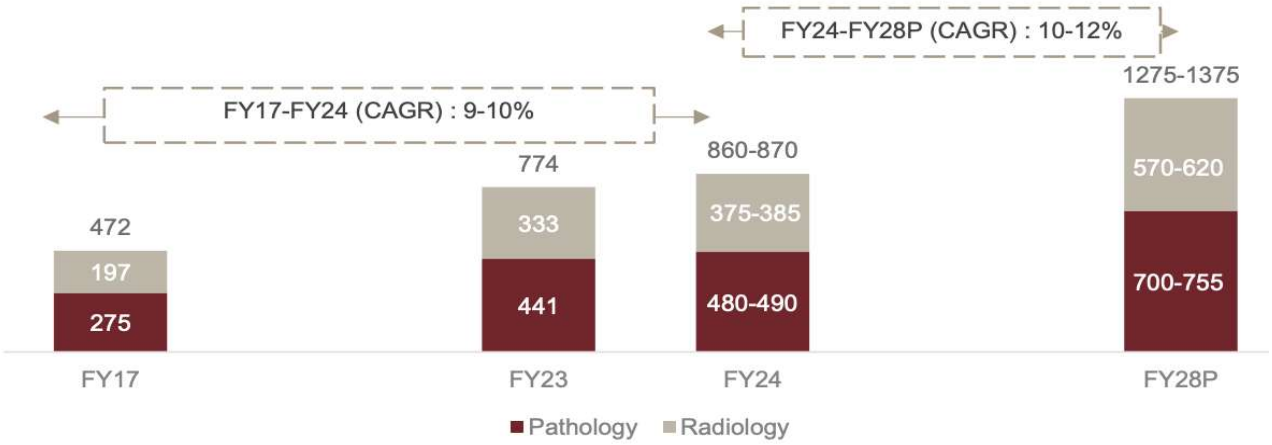

**Source:** Reedser Consulting, Chiratae Ventures and Amazon Web Services (AWS).

**Note:** F = forecast.

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**Exhibit 6: Indian Diagnostic Industry, Trend and Projection (₹ billion)**




*E: Estimated; P: Projected*

**Source:** CRISIL Market Intelligence and Analytics (MI&A), 2024.

**Notes:** E = estimated, P = projected.

The pathology segment, having grown at a compound annual growth rate (CAGR) of ~8–9% between FY2017 and FY2024, is projected to expand at 9–11% over FY2024–FY2028. Radiology, which grew at ~9.5–10.5% during FY2017–FY2024, is expected to accelerate further to 11–13% over FY2024–FY2028. These projections reflect sustained demand across both segments, supported by rising healthcare needs and technological advancement.